

CANONICAL LESSON

Maternal and Child Health: Support, Not Scoring

Decision-centred foundations for accountable sector intelligence

Production Unit	PU-B07-C23
Notebook	V7_B_N03
Status	Controlled educational draft v0.1
Data boundary	Synthetic instructional data only

Central decision: Use maternal and child health analytics to support timely human review and referral without scoring dignity, eligibility, or clinical worth.

Learning outcomes

Why this matters

Individual risk scores can stigmatize patients or silently substitute for clinical judgment and informed care.

The analytical workflow is useful only when evidence, uncertainty, authority, and action are connected explicitly. The model supports professional judgement; it does not replace accountable institutional authority.

Worked cross-sector example

A care-gap signal routes an authorized outreach review while the clinician retains diagnosis and treatment authority.

Boundary: The example supports learning and review. It does not constitute an official statistic, eligibility determination, enforcement action, clinical judgement, or operational authorization.

Practice and reflection

- 1.
- Add uncertainty for small denominators.
- 2.

- Protect privacy.
- 3.
- Separate access from quality.

Answer key

- 1.
- Use interval estimates and suppress/aggregate unstable cells.
- 2.
- Minimize fields, control access, aggregate geography, audit disclosure, and prohibit individual re-identification.
- 3.
- Distance/utilization describe access; guideline adherence, outcomes, experience, and safety describe quality.

Operational readiness checklist

- Purpose and decision owner are named.
- Evidence definitions, provenance, and lawful access are documented.
- Uncertainty and abstention are visible.
- Human review, escalation, monitoring, incident response, and retirement are assigned.
- Equity, accessibility, security, and sector-specific approval are complete.